MICHIGAN MEDICINE
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Treatment of Positive Donor Cultures

Guideline Overview, Purpose, and Scope

Deceased donors undergo blood, urine, and sputum cultures prior to organ procurement. Further, hospitals may perform microbiological testing of donors prior to brain death. Results of these tests may guide recipient treatment intended to reduce the risk of transmission of infection from donor to recipient. While blood cultures are relevant for all organ recipients, routine organisms found in sputum are only relevant to lung recipients, and routine organisms found in urine are only relevant to kidney recipients. The information below is intended as general guide for recipient treatment of positive donor cultures.

Communication of Donor Culture Results

1. See Transplant Center Post-Procurement Communication of Donor Results Guideline #32

Urine or Kidney Preservation Fluid (kidney recipients only)

1. *Staph aureus*, gram negative bacilli, *Candida* species and other more virulent or intrinsically resistant organisms (as determined by transplant ID)
    a. 14-day treatment course
    b. Oral route or long active IV route preferred if available
    c. Micafungin/rezafungin an option for *Candida* species likely to be fluconazole resistant (or if species unknown) (approved age 18 years or older)
2. Less virulent or resistant organisms
    a. 7-day treatment course

Sputum or BAL (lung recipients only)

1. Bacterial organisms from donor or recipient cultures
    a. 14-day treatment course
    b. Decisions made in discussion with lung transplant rounding team
2. Cystic Fibrosis patients
    a. Referral to transplant ID during evaluation period
    b. Treatment started as peri-operative prophylaxis optimally prior to start of procedure
3. Candida species resistant to routine posaconazole or recipient intolerant of posaconazole
    a. 14-day treatment course
    b. Micafungin or rezafungin (if discharged)
    c. Discussion with transplant ID regarding ongoing mold prophylaxis if posaconazole cannot be used

Blood Cultures (all recipients)

1. GPC in clusters without further identification
    a. Vancomycin or Daptomycin pending further information
2. *Staph aureus*, *Candida* species and more resistant or virulent organisms (as determined by transplant ID)
    a. 14-day treatment course
    b. Oral or long acting IV route preferred after discharge
    c. If donor with high-grade bacteremia or concern for endocarditis a longer course may be required
3. Most other organisms (e.g., non-MDRO gram negative organisms, *Streptococcal* species, coagulase negative *Staph*)
    a. 7-day treatment course
4. Coagulase negative staphylococcus (1 of 2) positive blood cultures from donor can be considered a contaminant

Positive Donor Strongyloides serology

1. Ivermectin 200 mcg/kg daily for two days, repeated at two weeks (comes as 3 mg tablets)

ANTIMICROBIAL STEWARDSHIP
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References	
	Citation

Revision History	
Date	**Revision**

The recommendations in this guide are meant to serve as treatment guidelines for use at Michigan Medicine facilities. If you are an individual experiencing a medical emergency, call 911 immediately. These guidelines should not replace a provider's professional medical advice based on clinical judgment or be used in lieu of an Infectious Diseases consultation when necessary. As a result of ongoing research, practice guidelines may from time-to-time change. The authors of these guidelines have made all attempts to ensure the accuracy based on current information, however, due to ongoing research, users of these guidelines are strongly encouraged to confirm the information contained within them through an independent source